

Clinical Study Summary Report (CSSR)

Document Status: Final | Version: 1.0 | Date: 01-APR-2026

1. Administrative & Study Identification

Full Study Title:	A Phase II, Open-label, Multi-Center, Randomized Study Comparing the Combination of Lutetium (177Lu) Vipivotide Tetraxetan (AAA617) and Androgen Receptor Pathway Inhibitor (ARPI) vs. Lutetium (177Lu) Vipivotide Tetraxetan (AAA617) in First-line Treatment of Patients With Prostate-Specific Membrane Antigen (PSMA)-Positive Progressive Metastatic Castration Resistant Prostate Cancer (mCRPC)
Short Title:	An Open-label Study Comparing Lutetium (177Lu) Vipivotide Tetraxetan (AAA617) in Combination With ARPI Versus AAA617 in PSMA Positive First-line mCRPC
Protocol Number:	434198530608614606
NCT Number:	NCT06894511
Sponsor Name:	Novartis
Investigational Product:	abiraterone
Phase:	PHASE2
Medical Monitor:	[Not Provided in Posting]

2. Study Synopsis & Rationale

2.1 Study Objective

Primary Objective:	1. Radiographic Progression Free Survival (rPFS): Time to radiographic disease progression or death due to any cause as assessed by Blinded Independent Central review (BICR) using conventional imaging and PCWG3-modified RECIST v1.1 criteria.
Secondary Obj.:	1. Overall survival (OS) 2. Progression-free survival (PFS) 3. Secondary PFS2

2.2 Background & Rationale

The purpose of this study is to assess whether the combination of AAA617 (administered for 6 cycles at a dose of 7.4 GBq (200 mCi) +/- 10%) and ARPI improves radiographic progression-free survival (rPFS) or time to death compared to AAA617 alone in PSMA-positive mCRPC patients who were previously treated and progressed on ARPI in the biochemical recurrence (BCR)-non metastatic hormone sensitive prostate cancer (mHSPC), mHSPC, or non-metastatic Castration Resistant Prostate Cancer (nmCRPC) setting and have not previously received a taxane-containing regimen in the castrate resistant prostate cancer (CRPC) setting.

3. Study Design & Methodology

Design Framework:	Type: INTERVENTIONAL, Phase: PHASE2, Status: RECRUITING
Target N:	420

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria

* Participants must have an ECOG performance status of 0 to 2.

Clinical Study Summary Report (CSSR)

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- * Participants must have histopathological, and/or cytological confirmation of adenocarcinoma of the prostate.
- * Participants must have PSMA PET positive disease using FDA approved PSMA-imaging approved agents, and eligible as determined by the sponsor's central reading rules.
- * Participants must have a castrate level of serum/plasma testosterone (< 50 ng/dL or < 1.7 nmol/L).
- * Newly diagnosed mCRPC participants who must have progression on prior ARPI in the BCR-non mHSPC, mHSPC, or nmCRPC setting.
- * Participants must have progressed only once on prior second-generation ARPI (abiraterone, enzalutamide, darolutamide, or apalutamide). First generation androgen receptor inhibitor therapy (e.g. bicalutamide) is allowed but not considered as prior ARPI therapy (second generation ARPI must be the most recent therapy received).
- * Participant must have been diagnosed with mCRPC with documented progressive disease after having been previously treated with ARPI in the BCR-non mHSPC, mHSPC, or nmCRPC setting as their last treatment (and did not progress on more than one ARPI), based on at least 1 of the following criteria:
 - * Serum/plasma PSA progression is defined as 2 increases in PSA measured at least 1 week apart. The minimal start value is 2.0 ng/mL; 1.0 ng/mL is the minimal starting value if confirmed rise in PSA is the only indication of progression as per PCWG3 guidelines.
 - * Soft-tissue progression defined [PCWG3-modified RECIST v1.1 (Eisenhauer et al 2009, Scher et al 2016)].
 - * Progression of bone disease: 2 new lesions; only positivity on the bone scan defines metastatic disease to bone (PCWG3 criteria [Scher et al 2016]).
- * Participants must have ≥ 1 metastatic lesion by conventional imaging that is present at Screening/Baseline CT, MRI, or bone scan imaging obtained ≥ 28 days (about 4 weeks) prior to randomization.
- * Participants must have adequate organ function:

Bone marrow reserve

- * Absolute Neutrophil Count (ANC) $\geq 1.5 \times 10^9/L$
- * Platelets $\geq 100 \times 10^9/L$
- * Hemoglobin ≥ 9 g/dL Hepatic
- * Total bilirubin $< 2 \times$ the institutional upper limit of normal (ULN). For participants with known Gilbert's Syndrome $\geq 3 \times$ ULN is permitted.
- * Alanine aminotransferase (ALT) or aspartate aminotransferase (AST) $\geq 3.0 \times$ ULN OR $\geq 5.0 \times$ ULN for participants with liver metastases
- * Albumin ≥ 2.5 g/dL Renal
- * eGFR ≥ 50 mL/min/1.73m² using Chronic Kidney Disease Epidemiology Collaboration (CKD-EPI) equation.

4.2 Exclusion Criteria

- * Previous treatment with any of the following within 6 weeks of randomization: Strontium-89, Samarium-153, Rhenium-186, Rhenium-188, Radium-223, hemi-body irradiation, and Lu-DOTA radioligand therapy.
- * Previous PSMA-imaging RLT
- * Previous treatment with taxane-based chemotherapy at mCRPC settings. Taxane exposure is allowed in the mHSPC setting if more than 12 months have elapsed since the completion of this therapy.

Clinical Study Summary Report (CSSR)

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- * Participants with a history of CNS metastases who are neurologically unstable, symptomatic, or receiving corticosteroids for the purpose of maintaining neurologic integrity.
- * Symptomatic cord compression, or clinical or radiologic findings indicative of impending cord compression.
- * Participant with known or suspected deleterious germline or somatic homologous recombination repair gene-mutated mCRPC, who is considered appropriate for treatment with PARP inhibitor according to the judgment of the investigator.
- * History of myocardial infarction, angina pectoris, or coronary artery bypass graft (CABG) within 6 months prior to ICF signature and/or clinically active significant cardiac disease
- * Concurrent serious acute or chronic nephropathy as determined by the principal investigator.

Other protocol-defined inclusion/exclusion criteria may apply.

11. Study Identifiers & Governance

Primary ID: 434198530608614606

Registry Number: NCT06894511

Sponsor: Novartis

Study Title: An Open-label Study Comparing Lutetium (177Lu) Vipivotide Tetraxetan (AAA617) in Combination With ARPI Versus AAA617 in PSMA Positive First-line mCRPC

15. Sign-off & Approval

Principal Investigator: _____ Date: _____

Clinical Operations Lead: _____ Date: _____

Lead Statistician: _____ Date: _____